

MedCore Hospitals & Healthcare Services Pvt. Ltd.

Hospital Billing Policy

Finance & Billing Department Policy

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Classification	Internal	Document Owner	Chief Financial Officer

1. Purpose

This policy establishes the principles, procedures, and controls governing the billing of patient services across all MedCore Hospitals facilities. It applies to inpatient, outpatient, diagnostic, day-care, and emergency services, and is intended to ensure that billing is accurate, transparent, timely, and consistent with the patient's admission category, treatment record, and applicable insurance or corporate arrangement.

The policy also defines the roles and approval authorities involved in generating invoices, applying discounts, coordinating with insurers and third-party administrators (TPAs), and resolving billing discrepancies raised by patients or their representatives.

2. Scope

This policy applies to all billing staff, cashiers, revenue cycle executives, insurance desk coordinators, department heads, and finance personnel involved in any stage of the patient billing lifecycle at MedCore Hospitals. It covers self-pay (cash) patients, insured patients, corporate/TPA-sponsored patients, and patients availing government or empanelled health scheme benefits.

This policy does not cover clinical decision-making, treatment protocols, or procurement of medical supplies, which are addressed in separate Clinical Operations and Procurement documents respectively.

3. Definitions

Term	Definition
Billing Cycle	The complete sequence of activities from service charge capture to final invoice settlement.
TPA	Third-Party Administrator engaged by an insurer to process cashless claims on its behalf.
Interim Bill	A provisional statement of charges generated during an ongoing inpatient stay.
Final Bill	The consolidated statement generated at the time of discharge, reflecting all chargeable services.
Cashless Authorization	Pre-approval issued by an insurer/TPA permitting direct settlement with the hospital.
Co-payment	The portion of the bill that the patient is contractually required to pay out-of-pocket under an insurance policy.
Write-off	An approved reduction of billed amount that is not recovered from the patient or the insurer.

4. Billing Principles

All billing activity at MedCore Hospitals is guided by the following principles, which billing staff must apply consistently regardless of the patient's payment category.

- **Accuracy:** Charges must reflect services actually rendered, as recorded in the clinical and pharmacy systems, and must not include duplicate or unverified entries.
- **Transparency:** Patients or their attendants must be able to obtain an itemized bill on request at any stage of treatment, not only at discharge.
- **Timeliness:** Interim bills must be updated at least once every twenty-four hours for inpatients, and final bills must be ready within the timeframes specified in the Patient Discharge SOP.
- **Segregation of Duties:** The staff member who generates a bill and the staff member who approves a discount or write-off on that bill must not be the same individual, except where explicitly permitted under Section 9.
- **Auditability:** Every adjustment, discount, or write-off must be logged in the Hospital Management System (HMS) with the approving authority's identifier and a stated reason.

5. Patient Categories

MedCore Hospitals bills patients under one of the following categories. The category assigned at registration determines the applicable workflow, required documentation, and settlement process.

Category	Description	Settlement Path
Self-Pay (Cash)	Patient or attendant settles all charges directly with the hospital.	Full payment at billing counter; interim deposits during stay.
Insured — Cashless	Insurer/TPA pre-authorizes and settles the eligible amount directly.	Cashless authorization request; co-payment (if any) collected from patient.
Insured — Reimbursement	Patient pays the hospital and separately claims reimbursement from the insurer.	Standard payment; hospital issues documents for the patient's claim.
Corporate-Sponsored	Charges billed to an employer or corporate account under an agreed tariff.	Corporate credit letter verified prior to admission.
Government / Empanelled Scheme	Charges billed under a government health scheme package rate.	Scheme ID verification and package mapping at registration.

6. Billing Process Workflow

The billing workflow begins at patient registration and continues through discharge. The stages below apply to inpatient episodes; outpatient and diagnostic billing follow an abbreviated version of the same sequence, omitting interim billing and deposit steps.

6.1 Pre-Admission and Registration

At the time of registration, the front-desk or admission staff captures the patient's payment category, insurance or corporate details (where applicable), and collects the initial deposit as specified in Section 8. The Patient Admission SOP governs the documentation required at this stage; this policy governs only the financial aspects.

6.2 Charge Capture During Stay

Charges for room occupancy, consultations, procedures, diagnostics, pharmacy, and consumables are posted to the patient's account in the HMS as they are incurred. Department staff are responsible for entering charges on the same day of service; charges older than seventy-two hours require a supervisor's note explaining the delay before they can be posted.

6.3 Interim Billing

For inpatient stays exceeding forty-eight hours, the billing desk generates an interim bill and shares it with the patient or attendant to support ongoing transparency and to surface any disputed charge early, rather than at the point of discharge when resolution options are more limited.

6.4 Discharge and Final Billing

On receipt of the discharge intimation from the treating physician, the billing desk prepares the final bill, reconciles it against interim bills already shared, applies any approved discount or insurance settlement, and coordinates with the Patient Discharge SOP to obtain billing clearance before the patient is discharged.

6.5 Post-Discharge Reconciliation

Where an insurer's final settlement differs from the pre-authorized amount, or where a corporate account requires later invoicing, the finance team reconciles the account within fifteen working days of discharge and issues a supplementary invoice or refund as required.

7. Service Categories and Indicative Rate Bands

Detailed tariffs are maintained in the Billing Rates and Limits workbook maintained by the Finance Department and updated periodically. The table below summarizes the broad service categories for reference; it is illustrative and does not replace the authoritative rate card.

Category	Examples	Billing Basis
Room & Nursing	General Ward, Semi-Private, Private, ICU	Per-day tariff, includes nursing care
Consultation	OPD, Inpatient rounds, Specialist referral	Per-visit or per-day tariff
Diagnostics	Laboratory, Imaging, Cardiology investigations	Per-test tariff as per service master
Procedures & Surgery	Minor procedures, Major surgery, Day-care procedures	Package or itemized, per procedure master
Pharmacy & Consumables	Medicines, disposables, implants	At issue price, itemized on bill
Ancillary Services	Ambulance, physiotherapy, dietician consultation	Per-service tariff

8. Invoice Generation and Tax Documentation

Every invoice issued by MedCore Hospitals carries a unique, sequentially generated invoice number that is never reused, along with the applicable tax registration details of the issuing facility. Interim bills are clearly watermarked as provisional statements so that they are not mistaken for the final tax invoice.

8.1 Itemization Requirements

Final invoices must itemize charges under distinct heads, including room and nursing, consultation, diagnostics, procedures, pharmacy, and consumables, rather than presenting a single consolidated figure. Package-rate procedures may show the package as a single line item, provided the components covered by the package are described in an accompanying annexure.

8.2 Applicable Taxes and Exemptions

Healthcare services that are exempt under applicable tax regulations are billed without tax; ancillary non-medical services such as television or telephone charges, where chargeable, attract tax at the prevailing rate and are shown as separate line items. The Finance Department is responsible for keeping the tax configuration in the HMS aligned with the latest applicable regulations.

8.3 Duplicate and Corrected Invoices

A duplicate copy of a previously issued invoice may be provided to the patient on request, clearly marked as a duplicate. A correction to an already-issued final invoice may only be made through a formal credit note and revised invoice, approved by the Finance Manager, and both documents are retained together in the patient's billing file.

9. Advance Payment and Deposit Policy

An advance deposit is collected at admission to cover anticipated charges during the course of treatment. The indicative deposit slabs are summarized below; treating physicians may request a revised deposit where the anticipated cost of treatment changes materially.

Scenario	Deposit Guidance
General Ward admission	Indicative package cost or a minimum standard deposit, whichever is higher
Semi-Private / Private admission	Higher of estimated package cost or applicable room-category deposit
ICU admission	Deposit revised daily based on evolving treatment plan
Planned surgery	Estimated package cost collected prior to the scheduled procedure date
Emergency admission	Deposit collected as soon as the patient is stabilized; treatment is never withheld pending deposit in a genuine emergency

Emergency care is never delayed or denied for lack of an advance deposit. This principle takes precedence over any deposit slab described above and is reinforced in the Emergency Department SOP.

10. Discount Approval Matrix

Discounts on billed charges may be granted only through the approval authorities listed below. Requests for discounts must be logged in the HMS with supporting justification before the discount is applied to the patient's account.

Discount Band	Approval Required
Up to 5%	Billing Supervisor
Discount Band	Approval Required
Above 5% and up to 10%	Finance Manager
Above 10% and up to 20%	Hospital Administrator
Above 20%	Chief Financial Officer

Discounts arising from an approved corporate tariff or an empanelled scheme package rate are not treated as discretionary discounts and do not require the approvals above, since they are pre-agreed contractual rates rather than case-by-case concessions.

11. Corporate and TPA Billing

For patients covered under a corporate agreement or an insurer's cashless network, the insurance desk verifies eligibility and obtains pre-authorization before non-emergency admission. Charges are billed to the insurer or corporate account up to the authorized limit; any amount beyond the authorized limit, or any item excluded under the policy terms, is billed directly to the patient.

Where a TPA raises a query or partially repudiates a claim, the insurance desk coordinates with the treating department to furnish clarifying documentation within the timeframe specified by the TPA. Amounts that remain repudiated after this process are billed to the patient in accordance with the cashless agreement signed at admission.

12. Billing Disputes and Resolution

A patient or attendant who disputes any charge on an interim or final bill may raise the concern with the billing desk at the time the bill is presented. The billing desk reviews the disputed line items against the clinical and service records within the same day, wherever possible.

Where the dispute cannot be resolved at the billing desk, it is escalated to the Finance Manager, who reviews the matter jointly with the concerned clinical department if needed. The escalation pathway and resolution timelines for billing disputes are aligned with the Patient Complaint Resolution Policy maintained by the Patient Services department, and billing staff should refer to that document for the full escalation matrix.

Disputes that remain unresolved after escalation to the Finance Manager are referred to the Hospital Administrator, whose decision is recorded in the patient's billing file together with the reasoning for the final determination.

13. Refunds

Where a review of a disputed charge, an excess deposit, or an insurance settlement results in an amount payable back to the patient, the refund is processed in accordance with the Patient Refund Policy. This billing policy governs only the identification and authorization of the refundable amount; the refund payment mechanism and timelines are detailed separately.

14. Interaction with Employee Reimbursement

Where an employee of MedCore Hospitals or an immediate dependent receives treatment at a MedCore facility, billing follows the same process as for any other patient of the applicable category. Any employee benefit or concession applicable to staff treatment is governed by the Employee Benefits schedule maintained by HR and is applied by the billing desk only on production of a valid staff authorization from HR.

15. Record Retention

Billing records, including interim bills, final bills, discount approvals, and insurance correspondence, are retained in the HMS and in the finance archive for a minimum of eight years from the date of the final invoice, in accordance with MedCore's Regulatory Compliance Manual. Access to archived billing records is governed by the Medical Record Access Policy and the Data Privacy and Confidentiality Policy.

16. Roles and Responsibilities

Role	Key Responsibilities
Billing Executive	Charge capture verification, interim and final bill generation, first-level query resolution
Billing Supervisor	Approval of discounts up to 5%, daily reconciliation of billing counter collections
Insurance Desk Coordinator	Pre-authorization requests, TPA correspondence, cashless claim tracking
Finance Manager	Discount approvals between 5% and 10%, dispute escalation review, monthly billing audit
Hospital Administrator	Discount approvals between 10% and 20%, final decision on escalated disputes
Chief Financial Officer	Discount approvals above 20%, policy ownership, periodic tariff review

17. Exceptions

Any deviation from this policy, including a discount, waiver, or billing timeline outside the parameters described above, requires written approval from the Chief Financial Officer and must be recorded in the exceptions log

maintained by the Finance Department. Emergency treatment is exempt from deposit-related provisions as stated in Section 9.

18. Frequently Encountered Billing Scenarios

The scenarios below illustrate how this policy applies in situations that billing staff commonly encounter. They are provided for guidance and do not override the detailed provisions in the sections above.

Scenario	Applicable Guidance
Patient is discharged before the insurer's final approval is received.	Hospital collects an interim deposit for the pending amount; the deposit is adjusted or refunded once the insurer's final settlement is received, per Section 6.5.
A corporate patient's treatment cost exceeds the corporate credit limit.	The excess amount is billed directly to the patient at the time of discharge, and the corporate desk notifies the employer's HR contact as per the corporate agreement.
A patient requests an itemized bill mid-treatment.	The billing desk generates an interim itemized bill on the same day, consistent with the transparency principle in Section 4.
A TPA partially repudiates a cashless claim after discharge.	The insurance desk raises the shortfall as a supplementary invoice to the patient after completing the clarification process described in Section 11.
A patient disputes a pharmacy charge on the final bill.	The billing desk cross-checks the charge against the pharmacy issue record within the same day; if unresolved, it is escalated per Section 12.

19. Related Documents

- Insurance Claims SOP (FIN-SOP-002)
- Patient Refund Policy (FIN-POL-003)
- Discount Approval Policy (FIN-POL-004)
- Employee Reimbursement Policy (FIN-POL-005)
- Patient Admission SOP (CLN-SOP-001)
- Patient Discharge SOP (CLN-SOP-002)
- Patient Complaint Resolution Policy (PS-POL-003)

20. Document Control and Revision History

Version	Date	Summary of Changes
1.0	01 April 2025	Initial release of the Hospital Billing Policy
2.0	01 April 2026	Revised discount approval matrix; added deposit guidance for ICU admissions; clarified TPA repudiation handling

Document Owner: Chief Financial Officer *Approved*
for internal circulation within MedCore Hospitals.